

Public Health Emergency Operations Center

"COUSP DRC"

MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°064/MVB_17/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (46)	• Haut-Uélé (4/13): Wamba, Boma Mangbetu, Pawa and Isiro • Ituri (27/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara and Mahagi. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (3/23): Makiso-Kisangani, Lubunga and Mangobo
Reporting date	July 17, 2026
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KEY INDICATORS — CUMULATIVE TO JULY 17, 2026

CASCONFIRMED — 5 PROVINCES 2,267* +86 new cases (Ituri75 · N-Kivu7 · Haut-Uélé4) 	CONFIRMED DEATHS · LETHALITY 893 · (39.4%)** +29 deaths (Ituri 25 · North Kivu 4) 	CASSUSPECTS OF THE DAY 236Ituri170 · N-Kivu61 · Tshopo1 · Haut-Uele 4 
PATIENTS IN ISOLATION 722 Ituri548 · N-Kivu174 	CURED — CUMULATIVE 412 +32 cured (Ituri32) 	NATIONAL CONTACT TRACKING 83.6% 9886/11,820 views · Ituri83.0% N-Kivu 87.0% - Tshopo 50.0% 

*National cumulative total as of 17/07: 2181 (16/07) + 86 new confirmations = 2267 cases, after retrospective consolidation.

**Overall lethality 39.4% (893/2267):.

1. Epidemiological Highlights

- **Coordination:** Visit by the Military Governor of Ituri to the Nizi Health Zone during the issuance of certificates to 10 patients cured of Ebola, with evaluation of the work of the CTE/CT and a reminder of the need for support from the Central Government in the provinces of Ituri, North Kivu and Tshopo.
- **Operational support :** Deployment of a national-level rapid response team in support of the system incident management in the Tshopo, Haut Uélé and Bas Uélé DPS.
- **Laboratory :** Reception of equipment at the HGR of the Logo Health Zone and continued strengthening of local diagnostic capacities in the Logo and Mambasa Health Zones.
- **Epidemiological situation :** No new health zones affected; the total remains at 46 affected health zones, including 45 entransmissionactive.
- **Case evolution:** In the last 24 hours, **86 new confirmed cases, 29 confirmed deaths and 32 recoveries have been recorded :**
 - o **Ituri :** 75 new cases, 25 deaths (14 community, 11 in CTE/CT) and 32 cures, bringing the total desguerisà406.

North **Kivu**: 7 new cases (Katwa 5; Butembo 2) and 4 deaths in Katwa; total of 214 cases and 130 deaths (lethality: 60.7%).

o **Haut-Uélé** : 4 new cases in Wamba; total of 22 cases and 13 deaths distributed across four health zones.

o **Tshopo**et**Sud-Kivu** : No new confirmed cases; Tshopo maintains 5 cases, 4 deaths and 1 recovered, while Sud-Kivu remains at 3 cases and 1 death, with 52 days without new cases in Miti-Murhesa.

- **Analysis** : Ituri remains the epicenter of the epidemic, concentrating **89.2%** of cumulative cases and **87.2%** of new confirmations; contact tracing performance remains insufficient and requires strengthening.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak in Bundibugyoebolavirus continued to evolve in a complex humanitarian and security context. Initially detected in the Mongbwalu health zone (Ituri), it spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), affecting 46 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continued to facilitate transmission. Multisectoral coordination remained mobilized to strengthen surveillance, intensify re-interventions, and limit the spread of the epidemic.



Figure 1 — Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

2.1 Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Case fatality rate, affected health zones, new cases (24 h)		
Ituri	2023	745	36.8%	27/36 (75.0%)	75
North Kivu	214	130	60.7%	11/34 (32.4%)	7
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Haut-Uélé	22*	13*	59.1%	4/13 (30.8%)	4
Tshopo	5	4	80.0%	3/23 (13.0%)	0
Total	2,267	893	39.4%	46 out of 140	86

* The data for the province of Haut-Uélé are being revised for harmonization and classification.

Ituri remains the epicenter of the 17th Ebola virus disease (EVD) outbreak, accounting for 89.2% of confirmed cases (n= 2,023) and 83.4% of deaths (n=745), with a case fatality rate of 36.8%. The main transmission hotspots remain the health zones of Bunia (606 cases), Rwampara (430), Mongbwalu (355), Nizi (168), and Nyankunde (99). In the last 24 hours, 86 new confirmed cases have been reported, including 75 in Ituri, 7 in North Kivu, and 4 in Haut-Uélé, all within the Bunia health zone.

(25), Lita (10), Nizi (9), Mongbwalu (8), Katwa (5), Bambu (5), Nia-Nia (5), Wamba (4), Fataki (3), Mambasa (3), Ariwara (2), Butembo (2), Tchomia (2), Aru (1), Kilo (1) and Mangala (1).

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	2023	745	36.83%	75	14	11	25
Bunia	606	197	32.51%	25	1	3	4
Rwampara	430	116	26.98%	0	0	3	3
Mongbwalu	355	196	55.21%	8	2	1	3
Nizi	168	79	47.02%	9	4	0	4
Nyankunde	99	28	28.28%	0	0	1	1
Mangala	56	33	58.93%	1	1	0	1
Lita	77	27	35.06%	10	1	0	1
Nia-Nia	47	12	25.53%	5	0	2	2
Bamboo	36	12	33.33%	5	1	0	1
Komanda	27	10	37.04%	0	0	0	0
Tchomia	21	5	23.81%	2	1	0	1
Unidentified	17	0	0.00%	0	0	0	0
Kilo	11	3	27.27%	1	1	0	1
Mandima	9	4	44.44%	0	0	0	0
Logo	7	1	14.29%	0	0	1	1
Aungba	6	2	33.33%	0	0	0	0
Damascus	7	0	0.00%	0	0	0	0
Mambasa	8	5	62.50%	3	1	0	1
Rimba	5	0	0.00%	0	0	0	0
Fataki	9	6	66.67%	3	1	0	1
Drodro	5	3	60.00%	0	0	0	0
Aru	4	1	25.00%	1	0	0	0
Kambala	2	2	100.00%	0	0	0	0
Gety	1	0	0.00%	0	0	0	0
Lolwa	2	1	50.00%	0	0	0	0
Boga	1	1	100.00%	0	0	0	0
Ariwara	6	1	16.67%	2	0	0	0
Mahagi	1	0	0.00%	0	0	0	0
North Kivu	214	130	60.75%	7	4		4
Katwa	92	67	72.83%	5	4	0	4
Butembo	55	27	49.09%	2	0	0	0
Blessed	34	24	70.59%	0	0	0	0
Musienene	14	3	21.43%	0	0	0	0
Kyondo	8	4	50.00%	0	0	0	0
Kalunguta	2	1	50.00%	0	0	0	0
Masereka	3	0	0.00%	0	0	0	0
Oicha	3	3	100.00%	0	0	0	0
Goma	1	0	0.00%	0	0	0	0
Vuhovi	1	1	100.00%	0	0	0	0
Mabalako	1	0	0.00%	0	0	0	0
South Kivu	3	1	33.30%	—			—
Miti-Murhesa	3	1	33.30%	0	0	0	0
Haut-Uélé	22	13	59.09%	4			0
Wamba	16	8	50.00%	4	0	0	0
Pawa	3	3	100.00%	0	0	0	0
Boma Mangbetu	2	1	50.00%	0	0	0	0
Isiro		1	100.00%	0	0	0	0
Tshopo	15	4	80.00%	—			—
MakisoKisangani	3	3	100.00%	0	0	0	0
Mangobo	1	1	100.00%	0	0	0	0
Lubunga		0	0.00%	0	0		0
Total	1 2 267	893	39.40%	56	18	0 18	29

2.3 Mapping of confirmed cases

The spatial distribution confirms a transmission mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwala, Nizie and Nyankunde, with a spread towards North Kivu (Katwa, Butembo and Beni), Haut-Uélé (Wamba) and Tshopo, reflecting a continuation of the geographical expansion of the epidemic.

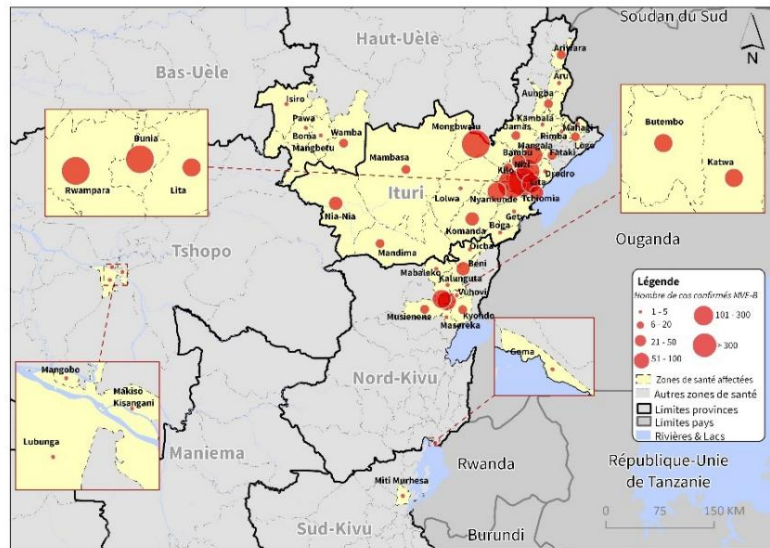
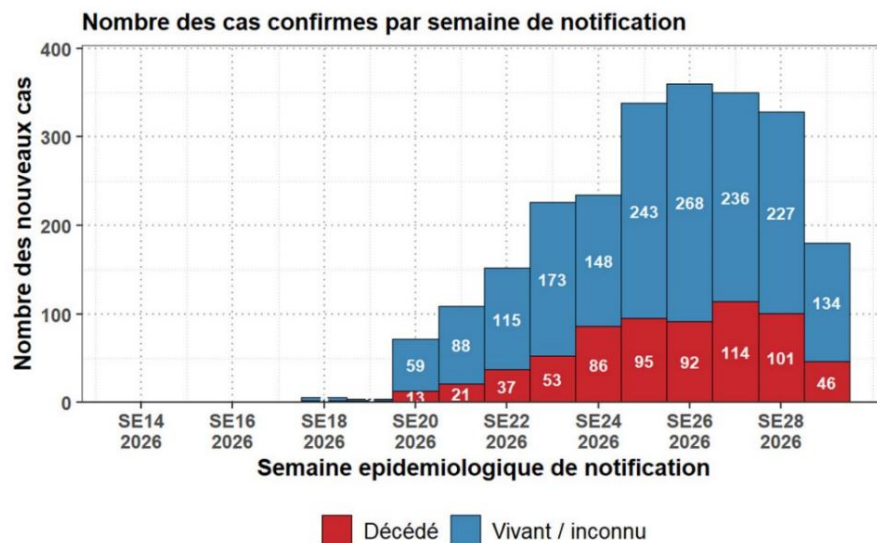


FIGURE 2 — HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES (AS OF 17/07/2026)

The recent emergence of new outbreaks in Haut-Uélé confirms a progressive expansion of the affected areas, highlighting the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in the neighbouring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained community transmission, with a continuous increase in confirmed cases during epidemiological weeks S25 to S28, each recording more than 300 cases.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses : cas confirmés avec statut vital et date de notification renseignés, n = 2 359.

FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

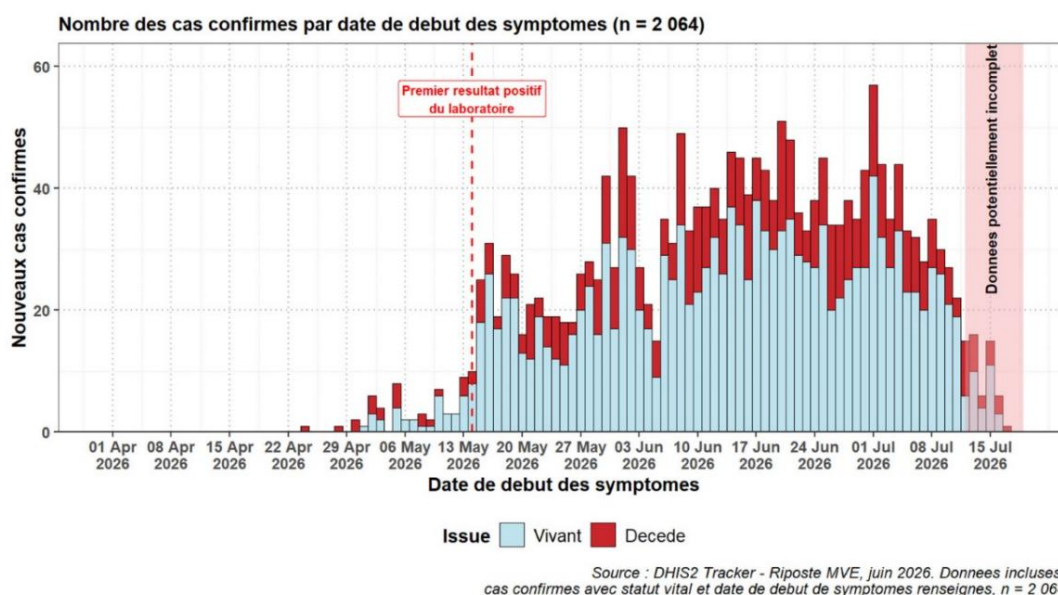


FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to notification delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults** (18–50 years). Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

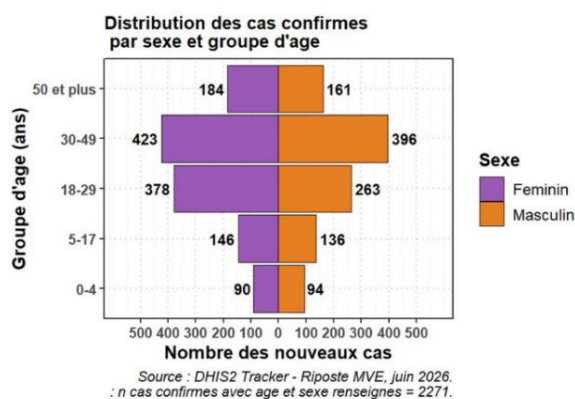
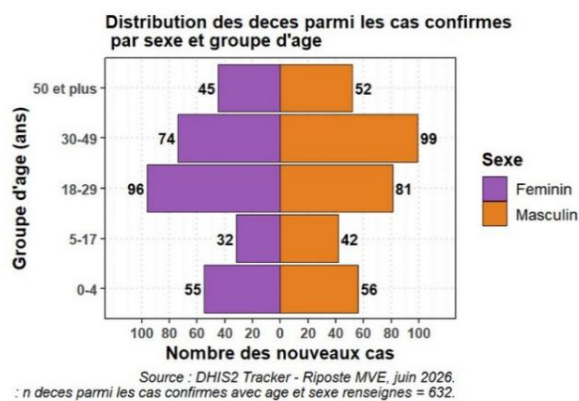


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG THE CONFIRMED CASES)



The distribution of cases according to sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS AS OF 17/07/2026

Indicator	Ituri North Kivu		Haut-Uélé	South Kivu	Tshopo	Total
Report (D-1)	5	4	35	ND	0	44
New alerts received	437	741	4	ND	1	1183
Total alerts for the day	442	745	39	ND	1	1227
Alerts investigated	402	741	4	ND	1	1148
Investigation rate (24 h)	90.9%	99.5%	10.3%	ND	100.0%	93.6%
Alerts confirmed — live	112	31	3	ND	1	147
Alerts confirmed — deceased (comm.)	58	30	1	ND	0	89
Total suspected cases for today	170	61	4	ND	1	236

Ituri: 442 alerts (437 new), 402 investigated within 24 hours (90.9%), and 170 confirmed (112 living cases, 58 community deaths). North Kivu: 745 alerts (741 new), 741 investigated (99.5%). Tshopo: 1 alert, 1 investigated (100%); 1 suspected case of the day.

3.2 Laboratory: tests and positivity

Ituri : 211 samples documented in the collection network, of which 75 were positive (61 living and 14 deceased), i.e., a apparent positivity of 35.5%.

North Kivu : 119 samples received and analyzed, of which 8 were positive, i.e. a positivity of 6.7%.

Haut-Uele : 4 samples taken and awaiting shipment for analysis. Four (4) new positive results. receipts from Nia-Nia's laboratory.

- The laboratory equipment intended for Logo has been received at the Logo General Hospital. The installation of diagnostic capacities in Mambasa and Logo remains planned to reduce confirmation times.

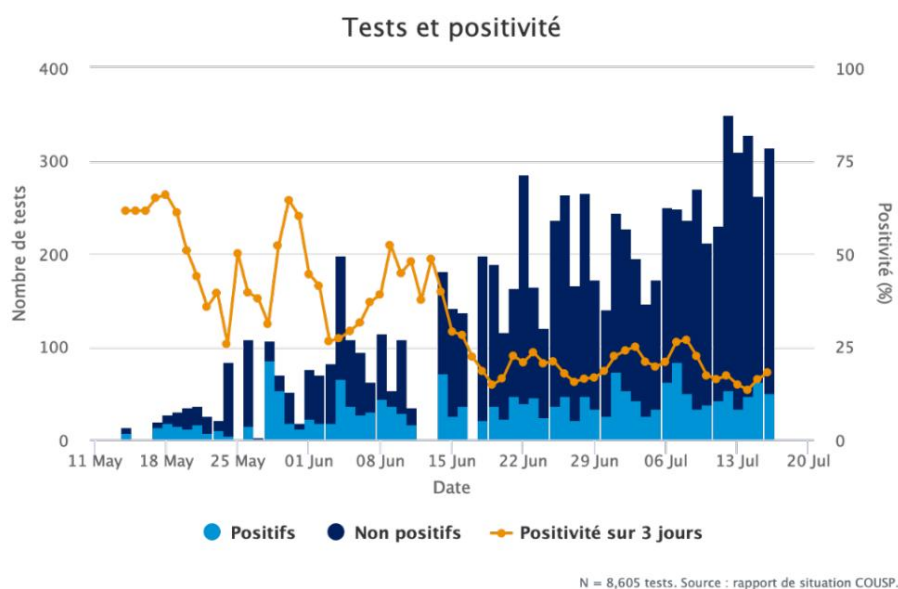


FIGURE 7 — TESTS AND POSITIVITY (COUSP SITUATION REPORT)

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	Follow-up rate
Ituri	8,660	7,192	83.05%
North Kivu	3008	2,618	87.03%
Tshopo	152	76	50.00%
Haut-Uélé	ND	ND	ND
South Kivu (end of monitoring)	0	0	N / A
Total	11,820	9,886	83.64%

Contact follow-up remains insufficient at the national level (83.6%), below the operational threshold of 95%. Ituri has completed 7,192 contacts out of 8,660 (83.0%), compared to 79% the previous day. North Kivu maintains a high performance with 2,618

Contacts seen in 3008 (87.60%). Follow-up is closed in South Kivu. However, contact follow-up figures are not available in the Haut-Uélé DPSH.

3.4 Entry points and control points (PoE/PoC)

In Ituri and North Kivu, 75 points of entry (PoE) and points of control (PoC) were operational (60 in Ituri and 15 in North Kivu). In total, 167,229 travelers were registered, of whom 160,400 (95.9%) were screened, 159,772 (95.5%) performed handwashing, and 165,032 (98.7%) received awareness training. Eleven (11) alerts were notified, of which nine (9) were validated. (8) were referred to care structures, while a lifeless body was intercepted in North Kivu. In Haut-Uélé, 9 of the 33 PoE/PoC were operational, with 4940 travelers checked, despite limited availability of supplies. In Tshopo, two (2) PoE/PoC remained operational and 797 travelers were screened, sensitized and directed towards handwashing.

3.4.1 — Crossing point alerts (24 h)

Ituri — 6 confirmed alerts

Six live alerts were detected at the Dele, Mudzibala, Makoko2 and Nzebi crossing points. All were validated;

Five cases were referred to a CT/CTE. Two instances of resistance or refusal to transfer required the involvement of community monitoring and communication teams.

North Kivu — 5 alerts (1 body)

Four (4) live alerts were notified and validated at the Posts of Control (PoCs) of Matombo, Eringeti, Mukulia and Kasitu; one lifeless body was intercepted at the PoC in Mukulia and referred to the organ of the Beni General Referral Hospital (HGR) for sampling. The 15 PoEs/PoCs of the province transmitted their reports (100% completeness) and 60,919 travelers were checked, for a screening refusal rate of 1.63%.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC), AS OF 17/07/2026

Indicators	Ituri	North Kivu	Overall
Proportion of activated PoC and reinforced PoE	100% (60/60)	100% (15/15)	100% (75/75)
Number of people switched to PoE/PoC	106310	60,919	167,229
% of travelers screened	94.5%	98.4%	95.9%
% of travelers who washed their hands	93.9%	98.4%	95.5%
% of travelers made aware	98.8%	98.4%	98.7%
Number of alerts notified	6	5	11
Number of validated live alerts	6	4	10
Number of bodies intercepted today	0	1	1
Number of problem contacts intercepted today	0	0	0

Indicators	Ituri	North Kivu	Overall
% of validated live alerts referred to the CT/CTE	66.7%	60%	63.6%
% of lifeless bodies	N / A	20%	20%
Number of positive results of referred suspected cases	0	0	0

Indicators PoE/PoC of July 17, 2026 (Ituri + North Kivu). Refusal of screening: 5855 travelers in Ituri and 974 in North Kivu; refusal of handwashing: 6483 in Ituri. The completeness/promptness of North Kivu was 93.3% (14/15 points), the PoC of Makeken not functioning due to insecurity.

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI), AS OF 17/07/2026

Indicator	Ituri	North Kivu, Haut-Uele, Tshopo			Together
Number of beds	700	141	ND	ND	841
Patients in bed (Day -1)	542	179	ND	ND	721
Total admissions (24 hours)	86	33	ND	ND	119
Total admissions	628	212	ND	ND	840
Deceased	19	0	ND	ND	19
No case	19	37	ND	ND	56
Outings	32	0	ND	ND	32
Escapes	10	1		ND	11
Transferred to the HGR	0	0		ND	0
Total outflows (24 h)	80	38		ND	118
Patients in isolation (end of day)	548	174		ND	722

Total, 722 patients were in isolation, including 252 confirmed cases and 460 suspected cases, for an overall occupancy rate of 85.9% (722 patients for 841 beds). Ituri is the main focus of the burden with 548 patients in isolation out of 700 beds, representing 78% occupancy, with 86 admissions and 80 discharges (32 recovered, 19 deceased, 19 non-cases, 10 escaped). In North Kivu, 174 patients are in isolation (17 confirmed, 155 suspected) with an occupancy rate of 123.4%.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY INDICATORS SMSPS, JULY 17, 2026

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	13 (76.4%)	1 (100%)
Confirmed cases currently being monitored	101 (68.7%) 16	2 (100%)
New cases suspected	(31.3%) 87	3 (100%)
Cassuspects in progress monitoring	(58.3%)	20 (100%)
Laboratory results announced (including positive ones)	38	8
Separated children—nursery/community	13	1
Accompanying persons at the CTE	0	0
Frontline staff (PPL)	0	0
Members of households and communities	114	4
EDSappuyés(SMSPS)	16	4

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD (CUMULATIVE AS OF JULY 17, 2026, ITURI)

Health Zone	Confirmed cases	Healed	In hospitalization	Death
Bunia	35	17	4	14
Rwampara	32	23	2	7
Mongbwalu	16	5	3	8
Nyankunde	14	11	1	2
Nia-Nia	10	0	10	0
Nizi	3	0	0	3
Mangala	3	1	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Lita	2	1	0	1
Total — case fatality rate 30.3%	119	61	22	36

PPL = healthcare personnel and frontline providers.

7 Pillar-Based Response Activities

COORDINATION Visit of the Military Governor of the Province of Ituri on the Occasion of the Ceremony of Discharge and Granting of Patents to 10 Ebola Victorious Patients Speaks to the Military Governor of the Province of Ituri in the Nizi Health Zone this Friday July 17, 2026 in the presence of the MCZ, the IMNational of the and the technical financial partners, response to the allocation of the central North Government intended for the three provinces of Ituri, Kivu and Tshopo. Evaluation of the construction works of CTE/CT in the health zone of Nizi. Arrival of the national COUSP team in support of the Tshopo incident management system and continued support for the affected provinces. Meeting held for the coordination to follow up on response activities, Reception at the Logo General Hospital of materials intended for the laboratory and continuation of the preparation of local diagnostic capacities in Mambasa and Logo.	PCI/WASH ÿ Ituri: 23/76 households/places of transit of cases decontaminated within 48 hours (30%), 6 health establishments and 6 means of transport disinfected; 175/1170 triage units operational. ÿ North Kivu: decontamination of one household and 6 health establishments; follow-up of 76 health establishments and briefing of 164 service providers, with insufficient kits in Oicha. ÿ Tshopo : training of 40 decontamination workers, assessment of the CHKoso (36%) and briefing of 48 service providers. ÿ North Kivu : decontamination of the household of a confirmed case in Butembo and of 6 ESSs of case stays (Butembo3, Katwa2, Kyondo1); provision of incomplete PCI kits in 4 ESSs of the Oicha health zone by CareInternational.
DIGNITE AND SECURE BURIAL (EDS) ÿIturi: construction of a decontamination base in Bunia and resumption of EDS in Nyankunde; resistance reported in Komanda and Mandima. ÿNorth Kivu: 47 death alerts, 47 bodies recovered/secured and 46 EDS carried out, with 11	Holistic Care (PECH) ÿIturi: 32 recovered, including 3 frontline staff and a 7-month-old infant; 86 admissions, 88 discharges and 54 severe/critical cases. ÿNorth Kivu: 173 patients hospitalized, including 17 confirmed cases, and capacity saturation; absence of

files awaiting consolidation. ÿ Haut-Uélé: 4 validated alerts, including one death, with documented transfer and sampling. %) then Oicha(6), Butembo(3), Musienene(1), Kalunguta(1) and Mabalako(1); the majority occurred in healthcare facilities.	CT/CTE in Musienene. ÿ Tshopo: 3 suspects isolated, monitoring of the repair of the tents and provision of 2 beds. ÿHaut-Uélé: 13 suspects reported to CT/CTE.
COMMUNICATION (RCCE/CREC) ÿIturi: 2538 people reached by home visits, 2177 by educational talks and 32035 by mass awareness campaigns; 296 community alerts and 88 documented feedback. Governor's involvement in Nizi and a priest in Mangala. ÿ North Kivu: briefings and dialogues in Beni, Kyondo and Manguredjipa. ÿ Tshopo: 1051 people sensitized, 540 leaflets, 65 posters.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ÿIturi: continued monitoring of affected frontline staff (see Table 8). No new cases reported on 17/07, the cumulative total remains at 119 cases and 36 deaths. Continued mapping of partners offering free care and monitoring of CapMEG; follow-up mission in the Rwampara health zone; monitoring of frontline staff, maintenance of the cumulative total of CSSE cases at 119 cases and 36 deaths, updating of the mapping of partners offering free care and continued monitoring.
RESEARCH, INNOVATION & VACCINATION ÿIturi: Closure of the training of CAP study investigators, continuation of anthropological studies in Mongbwalu and works vaccination protocols/EBO-PEP; meeting with UNAIDS and communities on Ebola-HIV co-infection. ÿTshopo: welcoming a research team to support specific care; receiving the UNAIDS Country Director as part of the response support; holding the weekly Research Pillar meeting bringing together national and provincial levels and partners; continued daily reporting of research monitoring indicators	LOGISTICS ÿ Ituri, delivery of kits to Nia-Nia, provision of PCI for traditional practitioners, allocation of 27 vehicles and 4 ambulances, consumption of 683.48L of diesel and 515L of gasoline, installation of 2 tents at the Nia-Nia CTE and completion of the Rwankole CTE. ÿ North Kivu : transport of medicines and civil protection supplies to Katwa, Beni, Butembo, Musienene and Oicha. ÿ Tshopo: 3 vehicles, 500L of water and cold chain equipment mobilized. ÿ Tshopo : logistical support for field teams and continued deployment of supplies
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA/PRSEAH) ÿIturi: Awareness-raising among parking attendants at Odoro, in Aru, on sexual exploitation and abuse (11 men affected) and among female vendors at the Gbanday market, in the Shari Health Zone (Rwampara Health Zone), affecting 50 people including 39 women, 1 man, 7 girls and 3 boys. Start of training of PSEA focal points in the Bunia Health Zones with the support of WHO, Save the Children and UNFPA; planning of the PSEA briefing for actors deployed in the Tshopo and Haut-Uélé Health Districts.	

8 Main challenges, impacts and actions required

Challenge No.	Impact	Actions required
1	Reluctance towards post-mortem sampling (EDS/swabs), community resistance and insecurity targeting EDS teams (attack on the bridge) Muchanga, paralysis of activities in Nyankunde)	Delayed confirmation, underestimation of cases, disruption of key activities Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the ZS of Bunia and Rwampara; resumption announced after	Interruption of post office activities, including dignified and secure burials Regularize payments to service providers; secure dedicated funding

Challenge No.	Impact	Actions required	
(the intervention of the military governor of Ituri)			
3	Insufficient CTE/CT capacity: occupancy of 126.2% in North Kivu (bed capacities not reported on 07/16, denominator renewed from 07/15) and 78% in Ituri (541 patients for 693 beds), including 51 severe or critical patients	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report capacities in beds.
4	Performance of contact tracing in strong degradation (Ituri 61.5% versus 78.1% on 15/07; Haut-Uélé 54.0%;Tshopo72.0%;national66.9%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs); strengthened supervision; investigate the observed decline in Ituri
5	Ambulance shortage and prepositioning at PoC	Delays in transfer and isolation	pre-position ambulances (including PoE/PoC), deploy CTs by ZS
6	Insufficient supply of ME/Medical inputs	Weakening of overall support	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited functional sorting units (175/1170, i.e., 15%)	High risk of nosocomial infections (119 cases of primary pulmonary encephalopathy, including 36 deaths)	Urgent supply, strengthening of PCI training
8	Geographic extension (Haut-Uélé, Tshopo) and strong mobility	Risk of urban, river and transboundary diffusion	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of local laboratories (Butembo, Mambasa and Logo)	Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
10	Financial shortfalls of the resources	Limitation of the implementation of pillars	Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restrictions on operations	Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increased indirect (non-Ebola) mortality	Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities



Presentation of certificates to 10 Ebola survivors by the military governor of Ituri province in the Nizi Health Zone



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